

Alternative format for summarizing the evidence- Evidence Profiles

Evidence profiles, an alternative format for presenting summaries of evidence, contain the same key information as Summary of Findings Tables (SoFs). Evidence profiles have additional columns with decisions for each of the domains of certainty of evidence and may have a column specifying the importance of each outcome.

№ of studies	Study design	Risk of bias	Certainty assessment				№ of patients		Effect		Certainty	Importance
			Inconsistency	Indirectness	Imprecision	Other considerations	intensive antileukemic therapy	less intensive antileukemic therapy	Relative (95% CI)	Absolute (95% CI)		
Mortality (follow-up: range 7.7 months to 60 months; assessed with: Risk of death at any point in time)												
11	non-randomised studies	not serious ^a	not serious	not serious	not serious	none	1014 participants	57.8% ^b 807 participants	HR 0.78 (0.69 to 0.89)	88 fewer per 1,000 (from 129 fewer to 42 fewer)	⊕⊕○○ Low	CRITICAL
Mortality (follow-up: 1 year; assessed with: Proportion of people who died)												
1	randomised trial	not serious	not serious	not serious	very serious ^c	none	22/44 (50.0%)	24/43 (55.8%) High risk	RR 0.83 (0.61 to 1.13)	95 fewer per 1,000 (from 218 fewer to 73 more)	⊕⊕○○ Low	CRITICAL
					serious ^d			20% Low risk		34 fewer per 1,000 (from 78 fewer to 26 more)	⊕⊕⊕○ Moderate	
Mortality (follow-up: 1 year; assessed with: Proportion of people who died)												
11	non-randomised studies	not serious ^a	not serious	not serious	serious ^d	none	509/978 (52.0%)	391/676 (57.8%)	RR 0.93 (0.85 to 1.01)	40 fewer per 1,000 (from 87 fewer to 6 more)	⊕○○○ Very low	CRITICAL
					not serious			20% Low risk		14 fewer per 1,000 (from 30 fewer to 2 more)	⊕⊕○○ Low	
Allogeneic hematopoietic stem cell transplantation (follow-up: range 8.5 months to 60 months; assessed with: Proportion of people who received transplantation)												
8	non-randomised studies	not serious ^a	not serious	not serious	not serious	very large effect ^e	172/747 (23.0%)	18/578 (3.1%)	RR 6.65 (4.13 to 10.71)	176 more per 1,000 (from 97 more to 302 more)	⊕⊕⊕○ Moderate	IMPORTANT
Complete remission (follow-up: range 13.3 months to 60 months; assessed with: Duration in months (or time to relapse))												
4	non-randomised studies	not serious ^a	not serious	not serious	serious ^f	none	There were 4 studies reporting this outcome. Most of them (3) reported a shorter remission with IC. The difference in CR duration ranged from 3.1 months shorter to 0.03 months longer when patients received IC versus less-intensive therapy.				⊕○○○ Very low	CRITICAL

Serious adverse events (follow-up: median 5 years; assessed with: Proportion of people who had Treatment emergent severe adverse events)

№ of studies	Study design	Certainty assessment					№ of patients		Effect			Importance
		Risk of bias	Inconsistency	Indirectness	Imprecision	Other considerations	intensive antileukemic therapy	less intensive antileukemic therapy	Relative (95% CI)	Absolute (95% CI)	Certainty	
1	non-randomised studies	serious ^g	not serious	not serious	serious ^h	none	59/95 (62.1%)	44/95 (46.3%)	RR 1.34 (1.03 to 1.75)	157 more per 1,000 (from 14 more to 347 more)	⊕○○○ Very low	IMPORTANT

Hospitalization (due to serious adverse events) (follow-up: range 20 months to 60 months; assessed with: Duration in days of hospitalization)

3	non-randomised studies	not serious ^a	very serious ⁱ	not serious	very serious ^j	none	217	153	-	MD 12.96 days higher (16.23 lower to 42.15 higher)	⊕○○○ Very low	IMPORTANT
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Quality of life - not reported

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CI: confidence interval; **HR:** hazard ratio; **MD:** mean difference; **RR:** risk ratio

Explanations

- a. Body of evidence from non-randomized studies, assessment of certainty of evidence started at low
- b. Used event rate from 1-year mortality for the less-intensive therapy (from non-randomized studies).
- c. Using a minimally important difference of 50 per 1000, the 95% CI suggests the possibility of an important benefit and an important harm.
- d. Using a minimally important difference of 50 per 1000, the 95% CI suggests the possibility of a trivial effect and an important benefit.
- e. The magnitude of effect is very large (point estimate 6.65)
- f. A meta-analysis was not possible, but it is likely that the pooled estimate would have crossed the null effect.
- g. The study was at serious risk of bias due to confounding and moderate risk of bias due to deviation from intended interventions so we rated down an additional level.
- h. Using a minimally important difference of 10%, the 95% CI suggests the possibility of trivial and important harm
- i. Not all confidence intervals of the studies overlap, and the statistical heterogeneity is high (I²=99%)
- j. Using a minimally important difference of 5 days, the 95% confidence interval suggests important benefit and important harm